

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 79 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: July 9, 2024 | Provider: James Okafor, MD — Nephrology | Doc ID: RFH-2024-0709-NEPH

Nephrology Consultation

CKD Stage G3b/G4 — Diabetic & Cardiorenal Disease

Reason for Consultation

Progressive decline in kidney function (eGFR 33), proteinuria, anemia, and electrolyte management in the setting of diabetes, hypertension, and heart failure.

Assessment

79-year-old with diabetic kidney disease and cardiorenal syndrome. eGFR has declined from ~70 (2015) to 33 (2024). Albuminuria category A3. Anemia of CKD present (Hgb 11.2). Secondary hyperparathyroidism noted on bone-health labs. Mineral metabolism and acidosis to be assessed.

Renal Panel

Test	Result	Units	Reference Range	Flag
Creatinine	2.1	mg/dL	0.7 - 1.3	H
eGFR (CKD-EPI)	32	mL/min/1.73 m2	> 60	L
Urine albumin/creatinine	640	mg/g	< 30	H
Potassium	5.1	mmol/L	3.5 - 5.1	H
Bicarbonate	20	mmol/L	22 - 29	L
Phosphate	4.6	mg/dL	2.5 - 4.5	H
Hemoglobin	11.0	g/dL	13.5 - 17.5	L
PTH, intact	96	pg/mL	15 - 65	H

- Plan (CKD Stage G3b/G4, A3)
- Maximize cardiorenal protection: continue empagliflozin (renal/HF benefit continues at this eGFR), sacubitril/valsartan and spironolactone with vigilant potassium monitoring.
 - Hyperkalemia (K 5.1): dietary potassium counseling; consider potassium binder if recurrent; review spironolactone dose with cardiology.
 - Metabolic acidosis: start sodium bicarbonate 650 mg PO BID (target bicarb >22).
 - CKD-mineral bone disease: dietary phosphate restriction; treat secondary hyperparathyroidism; vitamin D as per bone clinic.
 - Anemia of CKD: check iron studies; if iron-replete and Hgb <10, consider ESA — currently 11.0, monitor.
 - Metformin: reduce/discontinue if eGFR <30 — currently 32, reduce to 500 mg daily and re-evaluate.
 - Avoid nephrotoxins (NSAIDs, contrast caution). Discuss future renal replacement planning given trajectory and comorbidity; align with goals of care.
 - Nephrology follow-up in 3 months; labs in 4-6 weeks after changes.

Renal-Related Medication Changes

Medication	Dose	Route	Frequency	Indication
Sodium bicarbonate	650 mg	PO	Twice daily	Metabolic acidosis
Metformin	1000 mg → 500 mg	PO	Once daily (reduced)	T2DM — renal dose adjust
Empagliflozin	25 mg	PO	Once daily	Cardiorenal protection